

Health Intelligence Report

Patient: Sarah Chen
Generated: May 29, 2026

NOTICE: This report identifies patterns in health data to support informed conversations with a healthcare provider. It is not a medical diagnosis. Always consult a qualified clinician.

SUMMARY

Patient with hypothyroidism and resolved iron deficiency anemia shows stable vitals and appropriate specialist follow-up. Notable pattern: family history of autoimmune thyroid disease (mother's Hashimoto's) combined with patient's hypothyroidism warrants monitoring for autoimmune progression. Iron supplementation discontinued May 2026 after 4+ years suggests successful anemia resolution, but lack of recent ferritin/hemoglobin data creates uncertainty about current iron status.

PATTERNS IDENTIFIED

Potential Autoimmune Thyroid Disease Requiring Antibody Monitoring

Confidence: Possible pattern

RELATED CONDITIONS

Hypothyroidism

SUPPORTING EVIDENCE

- Mother diagnosed with Hashimoto's thyroiditis — Family History, Not dated
- Patient on levothyroxine 50mcg for hypothyroidism since May 2021 — Medications, May 2021
- Thyroid ultrasound performed — Radiology Appointment, November 2025
- Endocrinology follow-up scheduled for TSH recheck — Recent Appointments, June 2026

Iron Supplementation Discontinued Without Documented Resolution Labs

Confidence: Possible pattern

RELATED CONDITIONS

Iron Deficiency Anemia

SUPPORTING EVIDENCE

- Ferrous sulfate 325mg twice daily discontinued after 4+ years — Medications, May 2026
- Iron deficiency anemia diagnosed and managed since January 2022 — Conditions, January 2022
- Last hematology appointment for anemia management — Recent Appointments, February 2026

Vitamin D Supplementation Pattern Suggests Possible Deficiency or Thyroid-Related Need

Confidence: Possible pattern

RELATED CONDITIONS

Hypothyroidism

SUPPORTING EVIDENCE

- Vitamin D 2000 IU daily (weekdays) and 1000 IU weekends started — Supplements, May 2026
- Hypothyroidism diagnosis — thyroid dysfunction associated with vitamin D deficiency — Conditions, May 2021
- Timing coincides with discontinuation of iron supplementation — Medications/Supplements, May 2026

Type 2 Diabetes Family History With No Documented Metabolic Screening

Confidence: Possible pattern

RELATED CONDITIONS

Hypothyroidism

SUPPORTING EVIDENCE

- Mother and maternal grandmother both have Type 2 Diabetes — Family History, Not dated
- Hypothyroidism present — thyroid dysfunction increases diabetes risk — Conditions, May 2021
- Annual physical performed — Recent Appointments, April 2026

INFORMATION GAPS

- Most recent TSH, Free T4, and thyroid antibody (TPO/TgAb) levels to assess thyroid control and autoimmune status given family history
- Current CBC with ferritin and hemoglobin to confirm iron deficiency anemia resolution after stopping supplementation
- Vitamin D level (25-OH) baseline and target for ongoing supplementation
- HbA1c and fasting glucose results given strong family history of Type 2 Diabetes across two generations
- Thyroid ultrasound results from November 2025 to understand structural thyroid changes

TALKING POINTS FOR YOUR PROVIDER

- Has TPO antibody testing been done to confirm whether this is Hashimoto's thyroiditis given the family history, and should antibody levels be monitored periodically to assess disease progression?
- What were the final ferritin and hemoglobin levels that supported stopping iron supplementation, and what is the monitoring plan to ensure anemia doesn't recur?
- What was the baseline vitamin D level that prompted supplementation in May 2026, and is there a target level for monitoring given the thyroid condition?
- Given the strong maternal family history of Type 2 Diabetes and the presence of hypothyroidism, what is the current screening schedule for HbA1c and fasting glucose?

